

Pinky Promise – Investment deep dive

1. Company Overview

Company: Pinky Promise Private Limited

Brand: Pinky Promise

Founded: 2020; the current chat-first platform was established/launched in 2022

Headquarters: Mumbai, India

Founders: Divya Balaji Kamerkar (Co-founder & CEO) and Akanksha Vyas (Co-founder & CTO)

Directors: Divya Balaji and Rahul Anant Kamerkar

Sector: FemTech / Women's Health / HealthTech

Pinky Promise is an AI-assisted, doctor-led digital clinic focused on women's sexual and reproductive healthcare. It provides 24×7 chat-based access to gynaecological care, with consultations starting at ₹99 and coverage across 320+ symptoms/conditions. Rather than replacing doctors with AI, Pinky Promise uses AI to automate the time-intensive parts of a consultation—structured symptom collection, patient-history organisation, differential-diagnosis drafting and prescription drafting—while the doctor retains clinical judgment and approves the final medical advice. Importantly, the platform demonstrates relevance beyond urban, digitally privileged populations. Its asynchronous, chat-first model enables discreet consultations without requiring travel to a clinic, addressing infrastructural and socio-cultural barriers including distance, lost wages, limited privacy and stigma.

The platform has reportedly served 400,000+ women, with approximately 53% being first-time gynaecology users. It reports 97.4% three-month retention and approximately 40,000 paying customers.

2. Funding, Investors and Financial Performance.

Pinky Promise's most significant publicly reported financing is a \$1 million pre-seed round in January 2026, led by Rebalance Angel Community.

Reported investors/angels include Subramanian Ramadorai, Mala Ramadorai, Ajay Nair and Shreyas Srinivasan, alongside other senior executives/angels.

The company also received earlier grant/challenge support, including the FTxSDG Challenge, as well as support from organisations such as USAID, Villgro and Mulago Foundation.

FINANCIAL SNAPSHOTS:

₹ lakh	FY23	FY24	FY25
Operating revenue	0.91	9.97	9.42
Other income	~2.30	114.99	114.00
Total revenue	3.21	124.96	123.42
Total operating expenses	63.56	177.82	267.67
└ Employee expenses	22.38	98.80	137.52
└ Other expenses	39.49	61.80	98.94
Net loss	(60.40)	(54.25)	(145.25)
Debt	26.7	111	208

Pinky Promise's reported revenue jumped from ₹3.2L in FY23 to ₹1.25Cr in FY24, but remained broadly flat at ₹1.23Cr in FY25. However, the headline revenue is predominantly other income: ~₹1.15Cr in FY24 and ~₹1.14Cr in FY25, while operating revenue remained below ₹10L in both years. The nature of "Other income" was not visible with the financials in hand (assumed to be core income). At the same time, total operating expenses increased from ₹63.6L → ₹1.78Cr → ₹2.68Cr, with employee costs rising to ₹1.38Cr in FY25 and other expenses to ₹98.9L. Consequently, the company moved from a ₹54.3L loss in FY24 to a ₹1.45Cr loss in FY25, despite essentially unchanged total revenue. The financials therefore show a significant gap between reported revenue and underlying operating revenue, alongside a rapidly expanding cost base.

Ratios:

Ratio	FY23	FY24	FY25	What it shows
Net Profit Margin	-1,880.1%	-43.3%	-117.7%	Loss relative to reported revenue
EBITDA Margin	-1,861.2%	-40.3%	-106.3%	Operating profitability remains deeply negative
Current Ratio	3.39x	1.79x	0.61x	Short-term liquidity weakened materially

Debt / Equity	1.03x	-3.93x	-1.20x	Increasing leverage; negative equity makes this ratio difficult to interpret conventionally
ROIC	-114.8%	-65.4%	-423.5%	Capital employed is generating substantial negative returns

Cash Burn & Unit Economics

Pinky Promise reported ₹1.23 Cr revenue against ₹2.68 Cr expenses in FY25, resulting in an EBITDA loss of ₹1.31 Cr and indicating a significant funding requirement. (The cash flow statements were not present in the Financial reports obtained as of now to do detailed research)

Revenue per Customer: Revenue per paying customer cannot be reliably estimated from the available data, as Pinky Promise offers consultations at different price points, including **₹99 chat and ₹199 call consultations**, and the mix of services used by customers is not disclosed. Similarly, **LTV, CAC and LTV/CAC cannot be independently calculated** without customer-level utilisation, retention, churn and contribution-margin data. The key diligence question is whether repeat consultations, follow-ups and chronic-care conversion can generate sufficient contribution to support sustainable unit economics.

3. What could make Pinky Promise defensible ?

The potential moat is not simply “AI”. It is the combination of a specialised clinical dataset, proprietary workflow, doctor-in-the-loop architecture, patient behaviour data, privacy infrastructure and women's-health-specific clinical intelligence.

A. Purpose-built clinical AI architecture - One of Pinky Promise's strongest technical differentiators is its decision to separate language intelligence from clinical reasoning. The company uses LLMs for what they are good at—understanding messy, multilingual patient conversations and communicating naturally—but does not rely on an LLM alone for clinical decision-making. Its clinical layer is built around scientific evidence and clinical protocols, with the AI assisting doctors in systematically gathering information, structuring medical history and preparing differential diagnoses and prescriptions for review. This architecture attempts to address a fundamental problem with generic LLM-based healthcare: language fluency does not guarantee clinical consistency, protocol adherence or accountability.

B. Proprietary women's-health clinical intelligence

Pinky Promise says its AI is trained using 10,000+ sources and 250+ clinical protocols, alongside inputs from experienced gynaecologists and anonymised proprietary patient data. This is important because women's health remains comparatively underrepresented in medical research. The company highlights that women were only included in clinical trials in the US from 1993, creating historical gaps in women's-health datasets. The resulting opportunity is not merely to build a chatbot for women, but to develop clinical AI specifically optimised for female health conditions and presentation patterns.

Potential moat: women's-health-specific data → clinical protocols → model/workflow refinement → better consultations → more usage → more proprietary data

C. Proprietary consultation workflow

Pinky Promise has built its own clinical workflow rather than simply adding an AI chatbot to telemedicine. The AI handles structured symptom collection, patient-history organisation, asking follow-up questions, differential-diagnosis drafting and prescription drafting while the **doctor reviews and retains clinical responsibility**. This directly attacks the **administrative bottleneck** in gynaecology rather than trying to replace clinical judgment. Administrative automation has reduced documentation burdens, allowing doctors to focus on clinical judgement and counselling. Pinky Promise targets the repetitive work around **file creation, intake, prescription drafting and follow-ups**, potentially allowing the same clinical workforce to serve substantially more patients. The potential advantage is therefore doctor productivity.

The company has also designed the patient UI around easy, intuitive symptom disclosure, allowing AI to systematically ask questions and reduce the possibility of important symptoms being missed. Patients can upload prior records, which the platform's multimodal system reads and adds to the patient's medical file. On subsequent visits, the system uses historical conditions, family history, medications and contraindications to inform the current assessment—not merely store records.

The current clinical workforce comprises four full-time doctors working in shifts, supported by senior gynaecologists and an extended panel of 50–60 trained practitioners available for scale

Pinky Promise reports that doctors can currently “see” 250+ patients daily, with the company expecting capacity to increase further through its AI copilot, including automated management of follow-ups.

D. The "honesty gap" — a behavioural moat

One of the most interesting pieces of proprietary insight is that Pinky Promise found women were more forthcoming over chat than during verbal consultations.

Patients disclosed - embarrassing symptoms, medications they had taken, concerns they had kept private and information they had not mentioned during calls

This means chat is not merely a cheaper communication channel. The interface may improve the quality and completeness of the clinical information collected. That creates a behavioural advantage that generic telemedicine platforms may not automatically replicate. This is particularly relevant for rural and semi-urban users, where privacy at home, social stigma and travel to clinics can themselves be barriers to seeking reproductive healthcare.

E. RAG-based personalised copilot

Pinky Promise has built a RAG-based copilot that learns the communication style of individual doctors and patients. It attempts to preserve - personal context, continuity, language preference and communication style

This allows the system to communicate with patients in the language and style they are comfortable with, while retaining the clinical workflow underneath. The potential moat here is the accumulation of doctor-specific and patient-specific interaction context, making the platform progressively more personalised with usage.

F. First-contact user acquisition = untapped demand

Approximately 53% of Pinky Promise's users have never consulted a gynaecologist before. This is a significant traction metric because it suggests the platform is not merely shifting existing patients from offline consultations to telemedicine.

It is potentially creating a new healthcare access point for previously untreated/underserved women. The company reports that these are adult women who may have been managing conditions such as PCOS and recurrent infections for years without professional medical input. For an investor, this raises the possibility of a much larger addressable market than existing telemedicine users alone.

G. 97.4% three-month retention

Pinky Promise reports 97.4% three-month retention.

First-time gynaecology user → initial digital consultation → continued platform engagement. If independently validated, this could indicate that Pinky Promise is solving not only access, but also repeat engagement in women's healthcare. The approximately 30% share of users managing chronic conditions such as PCOS and endometriosis further supports the possibility of recurring rather than purely episodic engagement, as these programmes involve treatment plans, follow-ups and specialist referrals.

H. Privacy as product architecture

Privacy is not positioned merely as a policy; Pinky Promise says it was an architectural decision from day one.

The company states that - patient data is stored in-house, patient data does not leave its servers, training data is fully anonymised, personally identifiable and medical information is stripped before model training, records are encrypted at rest using AES-256, data is encrypted in transit using TLS 1.2 and only the consulting doctor has access to an individual patient's file

This is strategically important in reproductive and sexual healthcare, where privacy concerns can directly influence willingness to seek care and disclose sensitive information.

I. Affordability + potential operating leverage

Pinky Promise's pricing is a key part of its accessibility proposition.

The company provides free initial questions, while diagnosis and a treatment plan cost approximately \$1.20 (~₹99), with a 24-hour window for continued interaction with the doctor. According to Mulago Foundation's assessment, this is approximately 10% of the market price of a standard gynaecologist visit in India, while Pinky Promise's reported cost is approximately \$0.04 per consultation.

This creates a potentially significant gap between consumer price and delivery cost. If the AI-assisted workflow continues to increase doctor productivity, the combination of: low customer price + low marginal delivery cost + increasing consultations per doctor could provide the basis for scalable unit economics.

J. Evidence of outcomes — promising but still requiring validation

Pinky Promise reports that approximately 83% of health issues are resolved, based on self-reported user data. A woman had **severe, hospitalisation-level period pain for ~10 years**. Pinky Promise's doctors and nutritionist worked with her; the founder reports that within **6 months her periods became non-painful**, and within **8 months she conceived**, later having two children.

The company itself acknowledges that more rigorous evidence is required and has initiated a study with Emory University to evaluate the quality of diagnoses delivered through the platform.

K. Early Validation

The founders first tested the concept through **WhatsApp/Facebook and a basic chatbot**, before building the full product. Around **50% of users were routed through calls and 50% through chat**, enabling direct comparison of behaviour. The first **1,000 customers** were actively used for product feedback and iteration.

L. Distribution

Pinky Promise spent **₹0 on marketing for its first 1.5 years**, relying heavily on founder-led content, conversations and referrals. Divya personally spoke to early users and used educational Reels to acquire the first customers. This created an early **trust → positive experience → word-of-mouth** acquisition loop.

M. Trust & Privacy

Trust was treated as a core product problem, not merely a branding exercise. Consent, confidentiality and minimal initial data collection were built into the user journey; privacy concerns differ across segments, with Tier 2/3 users particularly worried about family discovering their consultations. The brand and product experience are deliberately designed around **safety, anonymity and trust**.

N. The ₹99 price point is deliberately designed around the affordability barrier

The founder says women can face **₹1,000–₹2,000** for a conventional gynaecology visit, which can cause care to be postponed. Pinky Promise deliberately reduced the entry price to **₹99**, making the first interaction low-risk and potentially converting previously untreated demand into paying healthcare users.

Pinky Promise identified that women leave consultations with many unanswered questions, so it introduced continued doctor chat after prescription. Users can extend the chat window for **₹59 after the initial 24-hour period**, creating a recurring micro-payment layer around continuity of care

O. Early-Stage Financing Challenge and Founder resilience

The company was **largely bootstrapped initially**, supplemented by grants and competition funding. Divya describes raising small amounts such as **\$2,000–\$5,000** to pay people building the technology, with cash-flow timing being one of the biggest early challenges. The key constraint was ensuring people who trusted the founders received their payments on time. Divya's earlier experience of **failing the UPSC twice**, including once by eight marks, shaped her ability to handle rejection. She connects this directly to fundraising, where many investors may reject a company before it eventually raises capital. She says this resilience helped her avoid being paralysed by rejection while building Pinky Promise.

P. Continuity is part of the product

The founder repeatedly distinguishes Pinky Promise from a one-time teleconsultation: the **prescription is the beginning, not the end**. Follow-up questions, treatment continuation and maintaining the patient's history in one place create the possibility of recurring engagement rather than a purely transactional consultation business.

Q. Female-healthcare behaviour itself can become proprietary knowledge

The founder's strongest insight is not simply “women need healthcare.” It is understanding **what prevents women from seeking it, what makes them disclose sensitive information, how privacy affects behaviour and what makes them return**. That behavioural layer can become strategically valuable when combined with the clinical dataset.

R. The emotional insight is commercially relevant, not just inspirational

Women often deprioritise spending on their own healthcare despite spending on other visible/urgent needs. The founder's thesis is that once a healthcare product **solves a clearly felt problem and earns trust**, willingness to pay and repeat usage increase. For Pinky Promise, this makes **trust-building a monetisation mechanism**, not merely a brand attribute.

S. International traction already emerging

Although India remains the core market, the founder says **~20% of the customer base was already international**, particularly across Sub-Saharan/West Africa, including Ghana, Nigeria, Senegal, Togo, Kenya, Tanzania and South Africa. This provides an early signal that the underlying women's-health problem may be geographically transferable

T. Rural penetration is already emerging without a separate rural product

The founder reports that women from **rural locations represented ~12% of users** over the preceding 3–4 months, while **70%+ of users overall came from Tier 2, Tier 3, Tier 4, smaller towns and rural areas**. This is important because the same chat-first product appears to be crossing beyond an exclusively urban customer base. Women in remote and low-connectivity regions, including parts of Jammu & Kashmir and the Northeast, access care through basic internet-enabled devices.

U. Pinky Promise – First physical clinic

Pinky Promise has launched its first physical clinic in Mumbai, expanding from digital to hybrid women's healthcare. The clinic focuses on a **women-centric, comfortable, and inclusive patient experience**. The clinic integrates with the Pinky Promise app for **doctor follow-ups and ongoing care beyond physical visits**.

4. Independent Guesstimate: Can Pinky Promise Scale to Profitability?

4.1 Objective & Key Assumptions

The objective is to independently test whether Pinky Promise can reach operating profitability through customer growth and operating leverage, rather than relying on management projections. The model starts with **~40K paying customers**, a consultation price of **₹99**, and assumes the current FY25 expense base of **₹2.68 Cr** as the starting cost base. Costs are assumed to increase by **5% annually** as the company scales, while technology and infrastructure investments are assumed to limit the need for proportionate cost expansion.

Assumption	Downside	Base	Upside
Current paying customers	40K	40K	40K
Incremental customers/year	10K	20K	30K
Consultations/customer/year	2	3	4
Price/consultation	₹99	₹99	₹99
Revenue/customer/year	₹198	₹297	₹396
Starting annual cost base	₹2.68 Cr	₹2.68 Cr	₹2.68 Cr
Annual cost growth	5%	5%	5%

The ₹2.68 Cr starting cost base represents FY25 total expenses reported for Pinky Promise Private Limited and should be treated as a modelling base rather than current consolidated operating costs, given the subsequent incorporation of Pinky Promise Healthcare Services Private Limited.

4.2 Five-Year Base Case

Under the base case, Pinky Promise adds **20K paying customers annually**, increasing from 40K currently to **140K by Year 5**. At three consultations per customer annually and ₹99 per consultation, annual revenue increases from approximately **₹1.78 Cr in Year 1 to ₹4.16 Cr in Year 5**. With costs growing at 5% annually, the model moves from a ₹1.03 Cr operating loss in Year 1 to approximately **₹0.74 Cr operating profit by Year 5**, with break-even reached around Year 4.

	Current	Y1	Y2	Y3	Y4	Y5
Paying customers	40K	60K	80K	100K	120K	140K
Revenue	—	₹1.78 Cr	₹2.38 Cr	₹2.97 Cr	₹3.56 Cr	₹4.16 Cr
Costs	—	₹2.81 Cr	₹2.95 Cr	₹3.10 Cr	₹3.26 Cr	₹3.42 Cr
Operating profit/(loss)	—	(₹1.03 Cr)	(₹0.57 Cr)	(₹0.13 Cr)	₹0.30 Cr	₹0.74 Cr

Market penetration: At 140K paying customers in Year 5, the model represents only **~0.039% of the ~360M women of reproductive age cited by Pinky Promise** — approximately 1 in every 2,570 women. This indicates that the modeled path to profitability does not require significant penetration of the company's cited addressable population; the key execution challenge is converting this theoretical market into paying, repeat users.

4.3 Downside / Base / Upside Sensitivity

The model is particularly sensitive to both **customer acquisition and consultation frequency**, while the ₹99 price remains constant. In the downside case, only 10K customers are added annually and each customer generates two consultations per year, leaving the company loss-making through Year 5. The upside case assumes 30K annual customer additions and four consultations per customer, resulting in substantially higher revenue and profitability.

	Downside	Base	Upside
Y5 customers	90K	140K	190K
Consultations/customer	2	3	4
Y5 revenue	₹1.78 Cr	₹4.16 Cr	₹7.52 Cr
Y5 costs	₹3.42 Cr	₹3.42 Cr	₹3.42 Cr
Y5 operating profit/(loss)	(₹1.64 Cr)	₹0.74 Cr	₹4.10 Cr
Break-even	Not reached	~Year 4	~Year 2

This sensitivity highlights that **customer growth alone is not sufficient**; utilisation of the platform is equally important. The model therefore tests whether reported retention can translate into repeated paid consultations, rather than assuming that retention automatically produces a particular consultation frequency.

4.4 Break-Even Customer Threshold

At the current ₹99 consultation price, each customer generates **₹297 annual revenue** under the base assumption of three consultations per year. Against the FY25 expense base of ₹2.68 Cr, approximately **90K paying customers** would be required to cover the existing cost base before accounting for cost growth.

As the cost base increases by 5% annually, the approximate break-even customer requirement rises from ~90K initially to **~115K by Year 5**. The base case crosses this threshold between Years 3 and 4.

Annual consultations/customer	Revenue/customer/year	Approx. customers required at ₹2.68 Cr cost
2	₹198	~1.35 lakh
3	₹297	~90K
4	₹396	~68K

This demonstrates the importance of **engagement and monetisation per customer**. Increasing consultation frequency can materially lower the customer scale required to reach operating break-even without increasing the consultation price.

4.5 Independent Model vs. Management DCF

The registered valuer's October 2025 report valued Pinky Promise Private Limited at **₹60.00 Cr as of September 30, 2025**, using a DCF based on **management-provided projected cash flows**. The DCF projects FCFF rising from approximately **–₹0.02 Cr in FY26 to ₹42.76 Cr by FY32**.

The independent model uses different assumptions around customer additions, consultation frequency and cost growth. Therefore, any difference between the two trajectories should be interpreted as a **sensitivity/diligence finding rather than a modelling error**. A significant gap would indicate that the documented valuation depends materially on management achieving substantially higher operating cash flows than the conservative independent case.

Key diligence question: Can the company achieve the customer growth, repeat utilisation and operating leverage required to bridge this gap?

4.6 Valuation Benchmark & Illustrative VC Return Sensitivity

The report concludes a **₹60.00 Cr fair equity value**, after arriving at an enterprise value of ₹69.95 Cr and applying adjustments and a 14% discount for lack of marketability.

Using ₹60.00 Cr only as a reference point, a hypothetical ₹5 Cr investment would imply approximately 7.7% post-money ownership before future dilution:

<i>Hypothetical Year-5 exit valuation</i>	<i>Value of 7.7% stake</i>	<i>MOIC</i>
₹50 Cr	₹3.85 Cr	0.8×
₹100 Cr	₹7.69 Cr	1.5×
₹150 Cr	₹11.54 Cr	2.3×
₹250 Cr	₹19.23 Cr	3.8×
₹500 Cr	₹38.46 Cr	7.7×

Illustrative sensitivity only; exit valuations are not forecasts and the analysis excludes future dilution, follow-on investments, transaction costs and fund-level considerations.

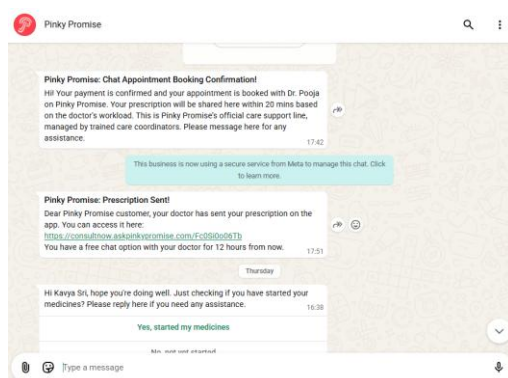
4.7 Investment Interpretation

The analysis points to an **operating-leverage thesis rather than a currently proven profitable model**. Under the base case, adding 20K paying customers annually takes Pinky Promise from 40K to 140K customers and towards operating profitability by Year 4, while representing only ~0.039% of the ~360M women of reproductive age cited by the company. However, the downside case remains loss-making, demonstrating that the model is highly dependent on customer acquisition and repeat utilisation.

The principal diligence questions are therefore whether **reported retention translates into recurring paid consultations, customer acquisition can remain efficient as the company scales, incremental costs remain broadly controlled, and revenue per customer can expand beyond the current ₹297 base-case assumption**. The gap between the independent model and management's DCF projections should be treated as an important area for further diligence rather than resolved by adjusting the independent assumptions.

5. First-Hand Product Test

To independently assess the customer experience, I personally purchased Pinky Promise's **₹99 consultation**. The experience provided a tangible validation of the company's chat-first care model. The chatbot guided me through multiple symptom and medical-history questions, while also allowing me to describe my concerns in my own words at the end of the interaction. A prescription was issued within approximately **20 minutes**, carrying the consulting doctor's official signature, followed by a **12-hour window for follow-up questions**. The WhatsApp interface also continued to follow up over the subsequent two days. From a product perspective, the experience demonstrated how Pinky Promise combines **structured AI-led information gathering, doctor-in-the-loop clinical decision-making and post-consultation engagement** within a ₹99 transaction. The combination of rapid prescription turnaround and proactive follow-up also suggests that the product extends beyond a one-time consultation toward an ongoing care relationship.



6. Competitive Landscape & Peer Benchmarking

Tracxn identifies several Indian women's-health platforms in Pinky Promise's competitive set, including **Newmi, Celes Care, Family, Pink Doctor, Gynic Care, Femease and Miyara Health**. However, most are private early-stage companies with **limited publicly available financial information**, particularly revenue, margins and profitability. **Peer benchmarking was therefore not undertaken**, as reliable and comparable financial metrics were unavailable across the identified competitors. The analysis instead focuses on Pinky Promise's **own historical financials, operating model and independent profitability scenarios**.

8. Why This Could Become a Strong Early-Stage Venture

There are **360 million women of reproductive age and approximately 70,000 gynaecologists in India**, highlighting a large demand-capacity gap. Conditions such as PCOS, fertility, infections and pregnancy concerns require recurring interaction rather than one-time consultations. The investment case is ultimately the possibility that Pinky Promise is building infrastructure for scalable women's healthcare, rather than simply another telemedicine application.

Its potential compounding loop is:

- 1) Underserved women enter through an affordable, private chat interface
- 2) AI collects richer and more structured clinical information
- 3) Doctors can handle more consultations because repetitive work is automated
- 4) More consultations generate proprietary, anonymised women's-health data
- 5) Data + clinical protocols improve the AI/workflow
- 6) Better personalisation and clinical experience drive repeat usage
- 7) Higher utilisation improves the economics of the doctor network

This creates multiple reinforcing assets: clinical data, workflow intelligence, doctor relationships, patient trust, behavioural insights and brand recognition in a highly underserved category.

Limitations & Key Risks

The analysis is based on **public disclosures, company-reported metrics and independent assumptions**, which may differ from actual operating performance. Customer growth, consultation frequency, pricing mix and cost assumptions are **illustrative and not management forecasts**. Limited disclosure on CAC, retention beyond reported periods, contribution margins and customer-level utilisation restricts the precision of unit economics. The model also does not fully account for **regulatory, clinical, competitive or execution risks**, which could materially affect growth and profitability.

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